SDG Goal 2	Zero hunger
SDG Target 2.2	By 2030, end all forms of malnutrition, including achieving, by 2025, the internationally agreed targets on stunting and wasting in children under 5 years of age, and address the nutritional needs of adolescent girls, pregnant and lactating women and older persons
SDG Indicator 2.2.2	Prevalence of malnutrition (weight for height >+2 or <-2 standard deviation from the median of the WHO Child Growth Standards) among children under 5 years of age, by type (wasting and overweight)
Time series	Wasting and Overweight

1. General information on the time series
   - Date of national metadata: July 6, 2023
   - National data: https://sdg-indicators.de/2-2-2
   - Definition: The time series shows the age-standardised prevalence of malnutrition among children aged 3 to 4 years. It is based on the population as at 31 December 2015.

     Wasting is defined as a weight-for-height ratio that is more than two standard deviations below the median of the World Health Organisation (WHO) growth standards for children.

     Overweight is defined as a weight-for-height ratio that is more than two standard deviations above the median of the WHO growth standards for children.
   - Disaggregation: Type of malnutrition, Sex
2. Comparability with the UN metadata
   - Date of UN metadata: March 2025
   - UN metadata: https://unstats.un.org/sdgs/metadata/files/Metadata-02-02-02a.pdf
   - The time series is partly compliant with the UN metadata. It covers only children aged 3 to 4 years and not the entire age group of 0 to 4-year-olds.
3. Data description
   - The data are taken from the „German Health Interview and Examination Survey for Children and Adolescents" (KiGGS). KiGGS forms part of the Robert Koch Institute's (RKI) health monitoring programme and comprises repeated surveys on the health status of children and adolescents. The study provides nationally representative data for the population under the age of 18.

     The first KiGGS baseline survey took place from 2003 to 2006 as a combined interview and examination survey. The first follow-up survey, KiGGS Wave 1, was carried out from 2009 to 2012. Data were collected via telephone interviews. KiGGS Wave 2 took place from September 2014 to August 2017. In addition to interviews, the study

programme also included physical examinations, laboratory analyses of blood and urine samples, and physical tests.

The data from the first KiGGS baseline survey (2003-2006) are based on a special evaluation. The examination component of KiGGS Wave 2 (2014-2017) included children and adolescents aged 3 to 17 years. Consequently, data on height and weight for the under-5 age group are available only for 3- and 4-year-olds (n=215 girls and n=221 boys).

4. Access to data source
   - German Health Interview and Examination Survey for Children and Adolescents (KiGGS): KiGGS Wave 2: https://www.rki.de/EN/News/Publications/Journal-of-Health-Monitoring/Issues/2018/JoHM_en_Inhalt_18_01.html?nn=16780210

5. Metadata on source data
   - German Health Interview and Examination Survey for Children and Adolescents (KiGGS): https://www.rki.de/EN/Topics/Noncommunicable-diseases/Health-surveys/Studies/KiGGS/kiggs_english_start.html

6. Timeliness and frequency
   - Timeliness: t + 12 months
   - Frequency: Irregular

7. Calculation method
   - Unit of measurement: Percentage
   - Calculation: Not applicable.